



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : حنان كامل محمد عبدالحميد عقل

العنوان : 9 ح عباس عبد الراضى

المحافظة : القاهرة الحي / المدينة : عين شمس

تاريخ تسجيل الإستمارة : 24-09-2026

رقم البطاقة : 27207020100449 رقم التليفون : 01232562154

العمر (تاريخ الميلاد) : 02-07-1972 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: حنان كامل محمد عبدالحميد عفا National ID: 27207020100449	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



07/21/2026



MDT FORM

Name: محمد عبد الحليم Kamel حاتم	MRN: 12021
Age: 54 Y	ID: 27207020100449
Menopausal Status: post	Phone: + 01012470503
NRS SCORE: ps 1	Marital Status: married
BMI: 35	Family History: -ve
Gender:	
CO Morbidity: hypothyroidism 100mg	Reason Of MDT: * NEW CASE
Surgical History: hemorrhoidectomy	ECOG Performance Status: 0 1 2 3 4

Clinical Data:

BX: IDC GII

ER 5/8 **PR** 5/8 **HER** 0 **KI67** 45



History:

SKIN EDEMA AND THICKNESS BY EXAM

Sonomammography:

CESM
UDQ MASS AND NON MASS ENHANCEMENT
10.5X 2.9X2.4 CM.

Biomarker/Molecular

Mets Workup:

CT CAP..... FREE

Recommendations:

FOR NACT THEN LT MASTECTOMY

MDT Outcome & Plan

FOR NACT (4 dose dense AC +4 CYCLES
DOCETAXEL) THEN LT MASTECTOMY

Right Side				Left Si	
Q1	Q2	Q3	Q4	Q1	Q2

Clinical Staging:

T4B N1 M0

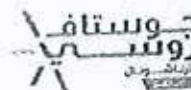
Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. محمد عبد الحليم	د. محمد عبد الحليم		



Gustave Roussy International-Egypt- Pathology Lab
جوستاف رومسي انترناتيونال - مصر - معمل الباثولوجي



Patient Name	حنان كامل محمد	Case No.	275399
Sex/Age	Female 53 Y	Patient No.	20512
Registered Date	08/6/2026 02:49:25 PM	Referred By Dr.	Interventional Radiology
Validation Date	25/6/2026 01:14:21 PM	Branch	Gustave Roussy
Reporting Date	30/6/2026 09:08:07 AM	Pathology No.	1799/26

Clinical Data: Left breast UOQ mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 08/06/2026, at 9:30 am. Cold ischemia time is 20 seconds.

Gross Examination:

- Fragmented grey, white tissue cores, ranging in size between 1.0 cm & 1.2 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 1)
- **DCIS:** Absent.
- **Microcalcification:** Absent.
- **Angiolymphovascular invasion:** Absent.
- **Perineural invasion:** Absent
- **Necrosis:** Focal.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 10% of tumor cells (score 5/8).
- PR: Strong nuclear reaction in 10% of tumor cells (score 5/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).
- Ki67: 45%.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 2.

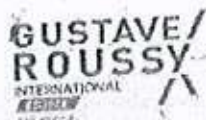
IPT:

- ER: Positive (score 5/8).
- PR: Positive (score 5/8).
- Her2: Ultra-low (score 0+).

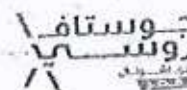


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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt

1/2026



Gustave Roussy International-Egypt- Pathology Lab
جوستاف رويسى انترناتيونال - مصر - معمل الباثولوجى



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- Ki67%: 45%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

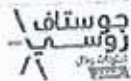
**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim.



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadima - Cairo, Egypt



Patient's Name: Hanan Kamel Mohammed Abdulhamid Akil
Date: 2026-07-15
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA.

I- CHEST:

- Left breast mass lesion measuring 33 x 30 mm with few small adjacent nodules and left axillary lymph nodes largest measuring 13 x 12 mm for sono-mammography.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Multiple subcentimetric mesenteric, para-aortic and bilateral external iliac lymph nodes.
- No ascites.

Opinion:

No pulmonary or hepatic metastatic lesions.

Dr.sherif Wagdy

Patient ID: 96992	Patient Name English: Hanan Camel Mohamed Abdallah
Age: 54	2026/07/07 01:45:31 PM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: left pathologically proved IDC III

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: minimal background enhancement

ACR: b (scattered fibro-glandular densities)

LEFT BREAST:

- Low energy (Non contrast) Images: UOQ posterior third spiculated irregular mass with anterior focal linear asymmetry .
- Recombined (Post contrast) images: UOQ posterior third moderate enhancing mass measures 2.9x2.9x2.4cm , linear faint enhancing non mass area extend from the lesion to about 12cm from the nipple.
- The overall coverage of enhancing area is 10.5x2.9x2.4cm
- Normal skin / nipple / areolar complex.
- Complementary US: 3:00 zone C irregular spiculated mass (index)
- Three satellites ranging 1.2x8cm & 0.6x0.7cm, the furthest located 2.8cm anterior to index lesion. And about 7cm from nipple .
- Pathologically enlarged level I lymph nodes, more than 3 all are discrete the largest is nearly round measures 1cm

RIGHT BREAST:

- Low energy (Non contrast) images No masses, asymmetry or distortion or suspicious microcalcifications.
- Recombined (Post contrast) images show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION:

LEFT BREAST: (BIRADS6 = pathologically proved malignancy) : UOQ pathological mass and non mass enhancement as described

RIGHT BREAST: (BIRADS 1= normal)

Best regards,

Dr. Marwa Ramzy.MD, FRCR

Name: Mrs. Hanan Kamel Mohamed

Date: 3/6/2026

ID:2226018468

Acc.:5259335

Referring Prof. Dr.:Ezzat Gabr.

Digital Mammography With Ultrasound Examination Of the left Breast, Revealed:

Indication: left breast palpable lump

Mammography:

- Scattered fibroglandular densities (ACR-b).
- Upper outer quadrant, middle and posterior thirds:
 - Irregular shaped spiculated margin heterogeneous dense lesion measuring 4.2 x 3.5 cm and about 16 cm distance from the nipple
 - More anterior to the lesion, similar but smaller lesions
 - The overall area of the lesions measuring about 8.3 x 3.5 x 2cm and about 13.4 cm distance from the nipple
 - The axilla showing few enlarged lymph node with thickened cortex and rather obliterated hilum
- No suspicious micro-calcific clusters noted.
- Normal skin thickness and contour. Normal areola / nipple complex.

Ultrasonographic examination showed:

- Non-homogeneous breast parenchyma with predominant fatty tissue
- Upper outer quadrant 2-3 o'clock irregular shaped spiculated heterogeneous hypoechoic solid lesion measuring 3.4 x 3.6 cm and about 0.7 cm distance deep to skin
- Anterior to the lesion at least two similar but smaller lesions, measuring from posterior to anterior
 - 12 x 13 mm, 13 mm distance from the index lesion and 10 mm deep to skin, and 8 mm distance from the third lesion
 - The other more anterior lesion measuring 12 x 8 mm, about 9 mm deep to skin and about 3 cm distance from the index lesion
 - The area in-between lesions showing altered parenchymal echogenicity
 - The overall pathological area measuring 7.7 x 3.6 cm
- No retro-areolar dilated ducts noted.

• **Regarding the axillary lymph nodes:**

- At least 5 level I axillary lymph nodes with globular shape and diffuse cortical thickness up to 7 mm and effaced hila
- Less than three level II lymph nodes with cortical thickness up to 4 mm

Opinion:

- Left breast: UOQ multifocal highly suspicious lesions as described (BIRADS 5)

Recommendation:

- Ultrasound guided biopsy from the left breast lesion
- Dynamic MRI breast for better assessment of the pathologic process extension

Dr. Lamiaa Elsharkawy, MD

BI-RADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- BI-RADS 0: Incomplete - need additional imaging evaluation or comparison with previous films.
- BI-RADS 1: Normal examination - nothing to comment on - Routine mammography screening is recommended.
- BI-RADS 2: Benign findings - routine mammography screening is recommended.
- BI-RADS 3: Probably benign <2% malignancy - short interval follow-up is suggested.
- BI-RADS 4: Suspicious >2 but <95% - Biopsy should be considered.
 - 4A: low suspicion of malignancy: 2-20%.
 - 4B: Mod suspicion of malignancy: 10-50%.
 - 4C: High suspicion of malignancy: 50-90%.
- BI-RADS 5: Highly suspicious of malignancy >95% - appropriate action should be taken.
- BI-RADS 6: Known - biopsy proven malignancy. Surgical excision when clinically appropriate.

American college of radiology (ACR) scoring of breast density:

- ACR. a: almost entirely fatty.
- ACR. b: scattered areas of fibro-glandular density.
- ACR. c: heterogeneously dense.
- ACR. d: extremely dense.